

1. Administrative & Study Identification

Full Study Title: DeniLA: Comprehensive Demographics and Clinical Profile of NSCLC Patients: Analyzing Guideline-Concordant Care in First-line Treatment Patterns
Short Title/Acronym: DeniLA
Protocol Number: D133FR00224
NCT Number: NCT07109154
Sponsor Name: AstraZeneca
Investigational Product: N/A (Observational Study / No Study Drug)
Phase of Development: Phase 4
Medical Monitor: AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate and analyze guideline-concordant care and patterns of choice in the first-line treatment of advanced (unresectable or metastatic) non-small cell lung carcinoma (NSCLC). **Secondary Objectives:** To understand the factors guiding the choice of first-line treatment regimens among thoracic and generalist medical oncologists and to compare these survey insights with real-world clinical data.

2.2 Background & Rationale To address the gap in understanding the demographics and clinical profiles of advanced NSCLC patients in real-world settings. This study aims to evaluate how first-line oncological treatments align with established guidelines and to capture the decision-making factors of oncologists in clinical practice across participating centers in Brazil.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Prospective and Survey) **Control Type:** Single-arm (Convenience sample) **Blinding:** Open-label (N/A for observational) **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$\$\$: 200 patients (and an estimated 200 filled survey files from oncologists) **Number of Sites:** 7 participating centers (in Brazil) **Estimated Study Start:** 25-Nov-2025 **Estimated Primary Completion:** 30-Apr-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age $\geq$ 18 years old. 2. Confirmed diagnosis of advanced (unresectable or metastatic) non-small cell lung carcinoma (NSCLC). 3. Receiving first-line oncological treatment in one of the participating centers between January 2025 and January 2026, with data available in electronic medical records.

4.2 Exclusion Criteria 1. Localized disease amenable to local treatment. 2. Non-epithelial histology, small cell carcinoma, or neuroendocrine tumor. 3. Patients with unresectable NSCLC who underwent treatment with the PACIFIC protocol and are being followed up.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: N/A (Observational study capturing real-world first-line NSCLC therapies) **Route of Administration:** N/A **Duration of Treatment:** N/A (Retrospective/prospective data extraction from medical records until study completion in April 2027)

5.2 Concomitant & Prohibited Therapy Permitted: Routine standard of care and first-line oncological treatments as prescribed by attending physicians. **Prohibited:** N/A (Real-world observational study).

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	Screening	N/A Electronic Medical Record (EMR) assessment to confirm eligibility
2	Baseline	N/A Data extraction of demographic and clinical profiles
3	Interim	N/A Survey distribution to thoracic and generalist medical oncologists
4	End of Study	30-Apr-2027 Final data collection, comparison of survey insights with real-world data

(Note: As this is an observational and survey-based study, traditional interventional clinical visits are not applicable. Data is extracted directly from electronic medical records.)

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Evaluation Patterns of Choice in First Line Treatment of Metastatic NSCLC, measuring guideline-concordant care in patient populations. **Measurement Method:** Extraction and analysis of clinical and demographic data from electronic medical records.

7.2 Secondary & Exploratory Endpoints Physician rationale and decision-making patterns guiding the choice of first-line treatment regimens, assessed via a targeted survey directed at medical oncologists.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: N/A (This is a non-interventional, observational chart review and physician survey; AE monitoring is subject to standard real-world pharmacovigilance rather than trial-specific monitoring). **Serious Adverse Events (SAEs):** N/A
Data Monitoring Committee (DMC): Managed internally by AstraZeneca for real-world evidence data integrity.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Convenience sample of 200 advanced NSCLC patients and approximately 200 surveyed medical oncologists.
Sample Size Justification: Target of $N = 200$ patients from 7 centers provides a representative real-world convenience sample for the demographic and clinical profile analysis in Brazil.
Handling of Missing Data: Excluded from primary complete-case analyses depending on electronic medical record availability.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP), observational research guidelines, and the Declaration of Helsinki. **Monitoring Plan:** Data quality checks on electronic medical record extractions and survey response validation. **Audit Trail:** Secure database logging of medical record extraction and anonymized physician survey responses.

11. Study Identifiers & Governance

Primary ID: {{D133FR00224}} **Registry Number:** {{NCT07109154}}
Sponsor/Lead Organization: {{AstraZeneca}} **Study Title:** {{DeniLA}} **Official Regulatory Title:** {{DeniLA: Comprehensive Demographics and Clinical Profile of NSCLC Patients: Analyzing Guideline-Concordant Care in First-line Treatment Patterns Observational Study (Prospective and Survey); Evaluation Patterns of Choice in First Line Treatment of Metastatic NSCLC}}

12. Clinical Framework (Study Specific)

Primary Condition: {{Lung Neoplasms / Advanced NSCLC}} **Diagnosis Threshold:** {{Advanced (unresectable or metastatic) non-small cell lung carcinoma}} **Medical Methodology:** {{Guideline-Concordant Care in First-line Treatment Patterns}} **Optimization Target:** {{Aligning real-world first-line oncological treatment choice with established clinical guidelines}}

13. Study Procedures & Methodology

Management Pathway: {{Real-world standard clinical pathway for advanced NSCLC}} **Education Protocol (HCP):** {{Survey targeting thoracic and generalist medical oncologists to understand treatment rationale}} **Education Protocol (Patient):** {{N/A - Observational EMR data extraction}} **Quality Audit Mechanism:** {{Comparison of self-reported physician survey insights with actual real-world patient data extracted from EMRs}}

14. Observation & Follow-Up Schedule

Total Duration: {{Observation period covers treatments initiated between January 2025 and January 2026, up to study completion in April 2027}} **Onsite Visit Cadence:** {{N/A - Data extraction from electronic medical records}} **Remote Monitoring Frequency:** {{N/A}} **Checklist Review Interval:** {{N/A}}

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				